



Lao People's Democratic Republic
Peace Independence Democracy Unity Prosperity

Child Protection Case Management Standard Operating Procedures



United For Every Child: 50th Anniversary of UNICEF in Lao PDR
ສາມາດຊີວິດແມ່ນີ້ຈຶ່ງ ເລີຍດັດຊະກຸງຄືນ: 50 ປີ ຂອງ ອົງການ ປຸ່ມໃຈຊື່ນ ແຫ່ງ ສປປ ລາວ



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Disclaimer: The photos in this document and the children shown within them are in no way associated with the subject matter of violence and child protection risks.

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Foreword

Child Protection Case Management Standard Operating Procedures (SOPs) are developed to implement the Law on the Protection of Rights and Interests of the Child to ensure that all children are fully protected. Helping vulnerable children who are experiencing or at risk of child protection issues requires a robust child protection case management mechanism for holistic and continuous service delivery, functional in both the development and emergency settings.

The development of this SOP has been coordinated with relevant sectors at the central and local levels, such as the Ministry of Justice, the Ministry of Education and Sports, the Ministry of Health, the Ministry of Public and Security, the People's Supreme Court, the office of Supreme People's Prosecutor, Lao Women's Union, the National Committee for the Advancement of Women, Mothers and Children, UNICEF Lao PDR, Friend International and other relevant international organisations. The Child Protection Case Management Standard Operating Procedures (SOPs) offer clear principles, codes of conduct, procedures, tools, roles, and responsibilities of each sector.

The Ministry of Labour and Social Welfare, as a chair of the Child Protection and Assistance Committee, would like to express our gratitude to all partners at the central and local level, international organisations, and experts who contributed to the development of this document. We appreciate the technical and financial assistance from UNICEF, the Korea International Cooperation Agency (KOICA), UNICEF Australia and the Australian Government through the Department of Finance and Trade (DFAT) to develop the Child Protection Case Management Standard Operating Procedures.

I sincerely hope that these SOPs are used by the social service workforce to provide children in need of help with well-coordinated and comprehensive responsive services through a systematic case management approach. The Ministry of Labour and Social Welfare will continue collaborating with all partners to develop a more effective child protection system to protect every child in the country better.

Vice Minister

Ministry of Labour and Social Welfare

Chair of the Committee of Child Protection and Assistance

ຄຳນຳ

ມາດຕະຖານຂັ້ນຕອນການຄຸ້ມຄອງກໍລະນີກ່ຽວກັບການປົກປ້ອງເດັກ ສະບັບນີ້ ສ້າງຂຶ້ນເພື່ອເປັນການຈັດຕັ້ງຜັນຂະຫຍາຍ ກົດໝາຍວ່າດ້ວຍການປົກປ້ອງສິດ ແລະ ຜົນປະໂຫຍດຂອງເດັກ ແນໃສ່ໃຫ້ເດັກທຸກຄົນໄດ້ຮັບການປົກປ້ອງຢ່າງເຕັມສ່ວນ. ການຊ່ວຍເຫຼືອເດັກທີ່ຕ້ອງການປົກປ້ອງພິເສດ ແລະ ເດັກທີ່ມີຄວາມສ່ຽງ ຈຳເປັນຕ້ອງມີກົນໄກການຄຸ້ມຄອງກໍລະນີທີ່ເຂັ້ມແຂງ ສາມາດເຮັດໜ້າທີ່ໃນການຄຸ້ມຄອງກໍລະນີທັງໃນສະພາບປົກກະຕິ ແລະ ໃນສະຖານະການສຸກເສີນ, ລວມສູນ ແລະ ຕໍ່ເນື່ອງ.

ການພັດທະນາມາດຕະຖານສະບັບນີ້ ໄດ້ມີການປະສານສົມທົບກັບພາກສ່ວນທີ່ກ່ຽວຂ້ອງ ທັງຂັ້ນສູນກາງ ແລະ ທ້ອງຖິ່ນ ເປັນຕົ້ນ: ກະຊວງຍຸຕິທຳ, ກະຊວງສຶກສາທິການ ແລະ ກິລາ, ກະຊວງສາທາລະນະສຸກ, ກະຊວງປ້ອງກັນຄວາມສະຫງົບ, ສານປະຊາຊົນສູງສຸດ, ອົງການໄອຍະການປະຊາຊົນສູງສຸດ, ສູນກາງສະຫະພັນແມ່ຍິງລາວ, ຄະນະກຳມາທິການແຫ່ງຊາດ ເພື່ອຄວາມກ້າວໜ້າແມ່ຍິງ, ແມ່ ແລະ ເດັກ, ອົງການ UNICEF ປະຈຳລາວ, ອົງການ Friend International ແລະ ອົງການຈັດຕັ້ງສາກົນອື່ນທີ່ກ່ຽວຂ້ອງ. ເພື່ອເຮັດໃຫ້ການຄຸ້ມຄອງກໍລະນີເປັນລະບົບຄົບຊຸດ ໄດ້ກຳນົດ ຫຼັກການ, ຈັນຍາບັນ, ຂະບວນການ, ເຄື່ອງມື, ພາລະບົດບາດ, ຄວາມຮັບຜິດຊອບຂອງແຕ່ລະຂະແໜງການໃນການຄຸ້ມຄອງກໍລະນີ.

ກະຊວງແຮງງານ ແລະ ສະຫວັດດີການສັງຄົມ ໃນນາມປະທານຄະນະກຳມະການປົກປ້ອງ ແລະ ຊ່ວຍເຫຼືອເດັກ ຂັ້ນສູນກາງ ຂໍສະແດງຄວາມຊົມເຊີຍມາຍັງພາກສ່ວນທີ່ກ່ຽວຂ້ອງ ທັງຂັ້ນສູນກາງ-ທ້ອງຖິ່ນ, ອົງການຈັດຕັ້ງສາກົນ, ຊ່ຽວຊານທີ່ໄດ້ທຸ່ມເທຫ້ອແຮງສະຕິປັນຍາ ແລະ ພິເສດ ຂໍຂອບໃຈມາຍັງ ອົງການ UNICEF, ອົງການ KOICA ປະຈຳ ລາວ ແລະ ລັດຖະບານ ປະເທດ ອົດສະຕຣາລີ ທີ່ໄດ້ໃຫ້ການສະໜັບສະໜູນດ້ານງົບປະມານ ເຂົ້າໃນການສ້າງມາດຕະຖານຂັ້ນຕອນການຄຸ້ມຄອງກໍລະນີກ່ຽວກັບການປົກປ້ອງເດັກ ສະບັບນີ້.

ຫວັງຢ່າງຍິ່ງວ່າ, ມາດຕະຖານຂັ້ນຕອນການຄຸ້ມຄອງກໍລະນີ ກ່ຽວກັບການປົກປ້ອງເດັກ ສະບັບນີ້ ຈະຖືກນຳໃຊ້ໂດຍບຸກຄະລາກອນສັງຄົມສົງເຄາະ ເພື່ອໃຫ້ເດັກທີ່ຕ້ອງການຊ່ວຍເຫຼືອ ໄດ້ຮັບການບໍລິການຄຸ້ມຄອງກໍລະນີທີ່ມີການປະສານງານ ຢ່າງເປັນລະບົບສົມບູນຂຶ້ນ. ພວກເຮົາຈະສືບຕໍ່ຮ່ວມມືກັບທຸກພາກສ່ວນ ໃນການພັດທະນາລະບົບປົກປ້ອງເດັກໃຫ້ມີປະສິດທິພາບຫຼາຍຂຶ້ນ ເພື່ອປົກປ້ອງເດັກທຸກຄົນ.

ຮອງລັດຖະມົນຕີ

ກະຊວງແຮງງານ ແລະ ສະຫວັດດີການສັງຄົມ

ປະທານຄະນະກຳມະການປົກປ້ອງ ແລະ ຊ່ວຍເຫຼືອເດັກ ຂັ້ນສູນກາງ



ລິປາວ ຢ່າງ



Acronyms

CPAC	Child Protection and Assistance Committee
CPN	Child Protection Network
CRC	Convention on the Rights of the Child
HIV/AIDS	Human Immunodeficiency Virus/Acquired Immune Deficiency Syndrome
Lao PDR	Lao People's Democratic Republic
LWU	Lao Women's Union
LYU	Lao People's Revolutionary Youth Union
MHPSS	Mental Health and Psychosocial Support
MOES	Ministry of Education and Sports
MOH	Ministry of Health
MOLSW	Ministry of Labour and Social Welfare
MOJ	Ministry of Justice
MOPS	Ministry of Public Security
PSC	People's Supreme Court
OSPP	Office of the Supreme People's Prosecutor
SOP	Standard Operating Procedures
UNICEF	United Nations Children's Fund

Key Definitions

Abuse is any act or omission that physically or morally harms a child, sexual abuse of a child, or incitement of a child to commit a crime or to do something that is physically or morally harmful.¹ Abuse includes all forms of physical, sexual, psychological or emotional ill-treatment.²

Abandonment means parents or guardians do not provide guardianship, care and upbringing.³

Alternative care refers to the care of children without parental care or at risk of being without parental care. There are a variety of different informal and formal types of alternative care, including residential and family-based. Not all types of alternative care are right for all children.⁴

Best interests of the child is a child rights principle, which means that adults must do what is best for the child. In all actions concerning children, whether undertaken by public or private social welfare institutions, courts of law, administrative authorities, or legislative bodies. The interests of children and young people should be thought about at all levels of society, and their rights should be respected by people in power.⁵

Best interest determination is identifying actions for children deprived of parental care, or at risk of being so, that are best suited to satisfying their needs and rights, taking into account the full and personal development of their rights in their family, social and cultural environments and their status as subjects of rights, both at the time of the determination of the child's placement in family-based or alternative care, and in the longer term.⁶

Case conference is a meeting for stakeholders that can (but does not have to) include the child, immediate family, extended family, and other professionals or community members involved with the child or family (e.g., teachers, doctors, nurses, counsellors, law enforcement, religious leaders, etc.).

Case is an individual or family that is receiving assistance/service to solve the issues related to child protection.⁷

Case management is the process of ensuring that an identified child has his or her needs for care, protection and support met. This is usually the responsibility of an allocated social worker who meets with the child, the family, other caregivers, and professionals involved with the child to assess, plan, deliver or refer the child and/or family for services and monitor and review progress.⁸

¹ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 15. December 2006

² Save the Children: <http://resourcecentre.savethechildren.se/node/2144>. Accessed December 2013.

³ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 5. December 2006

⁴ United Nations Guidelines for the Alternative Care of Children, 2009.

⁵ Article 3 of the UN Convention on the Rights of the Child, 1989.

⁶ United Nations Guidelines for the Alternative Care of Children, 2009.

⁷ Better Care Network Toolkit Glossary: <http://bettercarenetwork.org/sites/default/files/attachments/glossary.pdf>. Accessed May 2022

⁸ Ibid.

Case manager is the person who manages the cases.

Caseworker is a social worker who implements a case plan. A caseworker can be a professional or a para-professional depending on the case.

Child refers to anyone younger than 18 years.⁹

Child labour means children under the age of twelve who work in non-hazardous or hazardous places and that the child works for more than the working hours as stipulated under the labour laws and other relevant regulations.¹⁰

Child marriage is a formal marriage or informal union before 18 years of age.¹¹

Child offenders are children who are subject to criminal proceedings as suspects, defendants or convicted persons.¹²

Child protection is preventing and responding to abuse, violence, exploitation and neglect¹³.

Child protection system is defined as formal and informal structures, functions and capacities that have been assembled to prevent and respond to violence, abuse, neglect and exploitation of children.¹⁴

Children affected by HIV/AIDS means children who are infected with HIV/AIDS, children who have been half-orphaned or orphaned due to HIV/AIDS, or children living with an HIV/AIDS positive family member.¹⁵

Children with disabilities means children who have long-term physical, mental, intellectual or sensory impairments which may hinder their full and effective participation in society on an equal basis with others.¹⁶

Children without appropriate care are children who are not receiving suitable, continuous and quality care, nurture and guidance at a physical, emotional, social, and psychological level from either their families or from other primary carers who are meant to replace the family environment and who are responsible for their wellbeing and development¹⁷.

Children without parental care are children whose parents or guardians do not provide guardian-

⁹ United Nations Convention on the Rights of the Child. 1989.

¹⁰ Law on Labour (amendment), Article 2, point 11, Dec 2023

¹¹ Ministry of Labour and Social Welfare of Lao PDR. Core Recommendations for Strengthening the Child Protection System. 2022.

¹² Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 12. December 2006.

¹³ http://www.unicef.org/chinese/protection/files/What_is_Child_Protection.pdf. Accessed May 2014.

¹⁴ A Better Way to Protect ALL Children: The Theory and Practice of Child Protection Systems. Conference Report, UNICEF 2013
https://resourcecentre.savethechildren.net/node/7696/pdf/c956_cps_interi_or_5_130620web_0.pdf

¹⁵ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 10. December 2006.

¹⁶ The convention on the rights of persons with disabilities, 2006 <https://www.un.org/development/desa/disabilities/convention-on-the-rights-of-persons-with-disabilities/article-1-purpose.html>

¹⁷ Fact Sheet – Children Without Appropriate Care. Save the Children International, 2010.

ship, care and upbringing.¹⁸

Child safeguarding is the action taken to promote the welfare of children and protect them from harm. Safeguarding means protecting children from abuse and maltreatment, preventing harm to children's health or development, ensuring children grow up with safe and effective care, and taking action to enable all children and young people to have the best outcomes¹⁹.

Child victim of trafficking means any child who has directly suffered in his or her physical or mental health, dignity, freedoms or his/her property resulting from all forms of trafficking in persons.²⁰

Double-orphaned child is a child whose parents are both deceased.²¹

Early pregnancy means a girl before the age of 18 becomes pregnant.

Exploitation refers to using children for someone else's advantage, gratification or profit, often resulting in unjust, cruel and harmful treatment of the child. These activities are detrimental to the child's physical or mental health, education, and moral or social-emotional development. This covers manipulation, misuse, abuse, victimisation, oppression and ill-treatment.²² Manifestations include child labour, commercial sexual exploitation, child trafficking and child begging.

Family-based care includes kinship care, foster care and other forms of family-based care such as guardianship.²³

Foster care is a situation where children are placed by a competent authority for alternative care in the domestic environment of a family other than the children's family that has been selected, qualified, approved and supervised for providing such care.²⁴

Half-orphaned child is a child whose father or mother is deceased.²⁵

Institutional care is a group living arrangement for more than ten children, without parents or surrogate parents, in which care is provided by a much smaller number of paid adult carers. The same service is provided to all children irrespective of age, gender, abilities, needs and reasons for separation from parents. The service provision is often depersonalised, and care follows a strict structure and routine. Large residential schools or orphanages would be an example.²⁶

Neglect is a failure of parents or guardians to provide care, education, encouragement, and the

¹⁸ Ministry of Labour and Social Welfare of Lao PDR. Individual CPN Child Case Recording Book (draft).

¹⁹ Safeguarding children and child protection <https://learning.nspcc.org.uk>

²⁰ Ibid.

²¹ Ministry of Labour and Social Welfare of Lao PDR. Individual CPN Child Case Recording Book (draft).

²² Taken from Save the Children UK, Child Protection Training Manual Facilitator's Guide for Teacher Training, Southern Sudan, 2008.

²³ United Nations Guidelines for the Alternative Care of Children, 2009.

²⁴ Ibid.

²⁵ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 7. December 2006.

²⁶ Ministry of Labour and Social Welfare of Lao PDR. Guidelines on Alternative Care for Children without Appropriate Care in Lao PDR (draft).

conditions necessary for the development of children, such as health care, nutrition, safe shelter, education and moral development based on the family's economic conditions.²⁷

Out-of-school children means children who are of school age but are not in school.

Para-professional refers to one who has specialized knowledge and technical training and who works closely with and is supervised by a professional. A para-professional social worker is a group of supervised staff and volunteers – often community-based – which serves the needs of children and families, particularly where social welfare systems are underdeveloped or stretched. USAID/PEPFAR has defined para-social workers as receiving anything from a few days of training to up to 6 months of training. They are both paid and unpaid workers.²⁸

Physically abused children are children who have been beaten or tortured by their parents, guardians or other people.²⁹

Residential care is provided in a non-family-based group setting with paid/unpaid staff where some children live and receive care and are placed by a competent authority. It is an overarching term and can include a wide range of care settings, from small group homes to large residential facilities, such as infant homes and long-term baby care in maternity hospitals, children's homes and orphanages, institutions including hospitals and special schools/institutions for people/children with disabilities, special boarding schools where children are placed and cared for who have, for example, dropped out of school, engaged in risk practices, are victims of violence, are children from vulnerable families, have special educational needs, among other institutions and care settings.³⁰

Sexually abused children means children who have been raped or are victims of sexual relationships or obscenity.³¹

Sexual exploitation means any actual or attempted abuse of a position of vulnerability, differential power, or trust, for sexual purposes, including, but not limited to, profiting monetarily, socially or politically from the sexual exploitation of another.³²

Social service workforce refers to a variety of professional and para-professional workers serving in the social system. Just as the medical profession consists of doctors, nurses, physical therapists and technicians, the social service workforce comprises many cadres of people with various titles, roles and functions, but they all share a common goal - to care, support, promote and empower vulnerable people³³.

Violence is defined as the intentional use of physical force or power, threatened or actual, that re-

²⁷ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 4. December 2006.

²⁸ National Association of Social Workers. <http://www.socialworkers.org>

²⁹ Government of Lao PDR. Law on the Protection of the Rights and Interests of Children. Article 2, point 8. December 2006.

³⁰ Based on a working definition used by the Data and Analytics Section of, UNICEF New York (2019)

³¹ Ibid.

³² ST Secretary-General's Bulletin on Special Measures for the Protection from Sexual Exploitation and Abuse (ST/SGB/2003/13)

³³ United Nations Children's Fund. Guidelines to Strengthen the Social Service Workforce for Child Protection. UNICEF, New York, 2019.



sults or is likely to result in injury, death, psychological harm, maldevelopment or deprivation.³⁴

Street Children were categorized as either children on the street, who worked on the street and went home to their families at night; children of the street, who lived on the street, were functionally without family support but maintained family links; or abandoned children who lived completely on their own.³⁵

³⁴ United Nations, World Report on Violence Against Children, 2006.

³⁵ https://www.ohchr.org/sites/default/files/Documents/HRBodies/HRCouncil/RegularSession/Session19/A-HRC-19-35_en.pdf



Chapter I.

Introduction

i. Background

Most children and adolescents in Lao PDR are fortunate to grow up in caring families and communities, surrounded by immediate relatives, extended families, and community members who help to support and ensure their well-being. However, research shows that children in Lao PDR also face a range of risks and challenges related to protection. The National Survey on Violence Against Children 2014 found that 15 percent of women and 16.9 percent of men had experienced at least one form of physical violence before they were 18.

The Lao Social Indicator Survey II (LSIS II) 2017 showed that birth registration for children under the age of 5 was 73 percent, and only 10 percent of mothers or caregivers knew the birth registration process for their child. Regarding child labour, the survey found that 27.9 per cent of children between the ages of 5 and 17 worked in hazardous conditions, and 42.8 percent responded that they were involved in child labour the previous week.

In addition, child marriage remains a challenge in Lao PDR, with 16.3 percent of girls entering a marital union before they reach the age of 18. According to LSIS II, education was an important factor in early marriage. 46.4 percent of uneducated women between the ages of 20 and 49 were married before the age of 18, compared to just 1.5 percent of women in the same age group who had received higher education.

According to the Child Protection System Assessment 2021, the coordination mechanism and the division of responsibilities among the government agencies and CPAC appear to be irregular and fragmented. In addition, providing services and monitoring has not yet achieved its objectives and goal. Lack of a comprehensive Child Protection Information Management System (CPIMS) and lack of standardised data collection, research and evaluation on child protection affect the development of evidence-based interventions, policies and programs. The existing database of ‘children in need of special protection’ (from the central to the local level) is not systematic and is underutilised.

These SOPs were developed by the Department of Social Welfare of the Ministry of Labour and Social Welfare (MOLSW) in collaboration with the Secretariate of the Juvenile Justice Coordination Committee, Lao Women’s Union and the United Nations Children’s Fund (UNICEF) Child Protection team. Existing materials, such as the CPN Child Case Recording Book, Guidelines on Alternative Care for Children without Appropriate Care, Child Protection Network Code of Conduct, etc., were used to develop the SOPs. Useful resources from other countries have also been drawn on, analysed and informed to enrich the SOPs,

The Child Protection Case Management Standard Operating Procedures (CP Case Management SoP) also describes the definition of case management, general principles, role and responsibility, case management procedures, and tools/resources in handling cases of child protection, child rights violations and child maltreatment. Reviewing the SOPs every three years is suggested to ensure the practice, resources, and forms are up-to-date and resonate with the real, on-the-ground context.

ii. Legislations related to child protection

Legislations related to child protection is as follows:

- Law on Protection of the Interests and Rights of Children, no. 5/NA, dated 27 December 2006;
- Law on Preventing and Combatting Violence Against Women and Children, no. 56/NA, dated 23 December 2014;
- Law on Anti-Trafficking in Persons, no. 73/NA, dated 17 December 2015;
- Law on Juvenile Criminal Procedure, no. 41/NA, dated 20 December 2013;
- Penal Code, no 26/NA dated 17 May 2017;
- Law on Persons with Disabilities, no. 57/NA, dated 10 December 2018);
- Labour Law, no. 43/NA, dated 24 December 2013;
- Criminal Procedure Law (Amended version), no. 37/NA of 14 November 2017;
- Education Law, no.62/NA dated 16 July 2015;
- National strategy on social protection, no 224/PM, dated April 1, 2020;
- National Plan of Action for Preventing and Combatting Trafficking in Persons (Phase I (2013-2015) No. 37/PM, dated 03 February 2014) and Phase 2 (2017-2020), no. 49/PM, dated January 23, 2018, and phase III (2021-2025) No. 027/CATIP, dated 23 August 2021
- National Plan of Action for Prevention and Elimination of Child Labour 2021-2025;
- National Plan of Action for Child Protection System Strengthening 2022-2026.

iii. Purpose

The purpose of this Child Protection Case Management Standard Operating Procedures is to guide the understanding of case management procedures, including identifying and referring child cases and acting as a reference for social workers, village child protection networks, and other organisations that work directly with children.

This Child Protection Case Management Standard Operating Procedures is used for coordination, assistance and referral between the social welfare sector, justice, health, education, public security and Lao Women's Union.

iv. Definition of Case Management

Case management is the process of ensuring that an identified child has his or her needs for care, protection and support met. This is usually the responsibility of an allocated social worker who meets with the child, the family, any other caregivers, and professionals involved with the child to assess, plan, deliver or refer the child and/or family for services, monitor and review progress.³⁶

³⁶ Better Care Network Toolkit Glossary (<http://bettercarenetwork.org/sites/default/files/attachments/glossary.pdf>)



Case management is offered throughout the life of a child protection case, from the initial investigation to case closure. Case management involves working with families to establish goals, creating plans to achieve the goals, providing services to meet needs identified in assessments, monitoring progress toward achievement of the goals, and closing cases when the goals have been achieved. It is crucial that child protective services staff acknowledge that each family is different and aim to provide tailored, culturally relevant services.

v. Categorisation of Child Protection Cases

According to the MOLSW criteria for vulnerable children, the following categories of child protection cases can be addressed using the SOPs.

- Abandoned children
- Children affected by HIV/AIDS
- Children in contact with the law, including offenders, victims and witnesses
- Child labourer
- Children married before the age of 18 years
- Child victims of trafficking
- Children with disabilities
- Double-orphaned children
- Girls pregnant before the age of 18 years
- Half-orphaned children
- Neglected children
- Out-of-school children
- Physically abused children
- Sexually abused children
- Street Children
- Children and drug addiction
- Others

Please see Chapter III. Tools and Forms for more information about the inter-sectoral referral pathway for these categories of child protection cases.

vi. Guiding Principles

The Core Recommendations for Strengthening the Child Protection System in Lao PDR (2021) define the general principles and values underpinning the child protection system. They are:

- Best interests of the child based on the CRC: In recognition of the Convention on the Rights of



the Child (CRC) and standards related to child protection, the child protection system in Lao PDR will uphold the principle of the best interests of the child being a primary consideration in all decisions affecting the child. The application of this principle will also take into consideration the social-cultural context of Lao PDR.

- Promote harmony within/among the family and place family strengthening at the centre of efforts to ensure the well-being and protection of children: The formal child protection system should support and strengthen the central role played by parents and extended families in ensuring child well-being. There will be situations where circumstances will require more formal responses as mandated by the legal and regulatory framework. Children should be kept together with families whenever possible, and institutionalisation should be the last resort.
- Empowering children, families and communities to be active partners in the child protection system: Services should be provided to support and work in partnership with families and communities. Families and communities have clear views on how child protection and well-being can be addressed, and they are an invaluable resource.
- Ensure that the services are understood and seen as beneficial by children, families and communities: The child protection system and services should reflect the aspirations and needs of children, families and communities. Services should aim to strengthen rather than replace positive community practices. These services should support the protective role that parents, families and elders play in ensuring the well-being of children.
- The system needs to be responsive and able to adapt to changes in the context in which children, families and communities live. Socioeconomic trends, migration, technology, social media and other factors are causing significant shifts impacting the lives of Lao children and families, including protection concerns. The child protection system should monitor and be able to effectively respond to different issues to ensure the well-being of children.
- Non-discrimination: Children should not be discriminated against because of gender, age, ability, wealth, migration status, parental status, etc. The case management process should be inclusive for all children.
- Confidentiality: Anyone involved in case management must treat any personal information that s/he has access to, whether about a child, other individuals who report their concerns, alleged perpetrators of violations or one of their colleagues, as confidential and not share it with third parties who do not have a legitimate reason for having access to it.
- Respect and collaboration: Caseworkers should treat other members or individuals who bring their concerns with respect and work with them in a spirit of cooperation. Given the cross-cutting nature of child protection, caseworkers should also work with other sectors, organisations, civil society associations, etc., to deliver holistic case management services.
- Child participation: Throughout the case management cycle, a child must meaningfully participate and should be given opportunities to express his/her needs and wishes. The caseworker

should always seek informed consent.

- **Prevention of sexual exploitation and abuse:** Any attempted and/or actual act or form of sexual harassment that could result in physical, sexual or psychological harm or suffering to individuals, especially women and children, is strictly prohibited. Sexual exploitation or abuse of beneficiaries under any circumstances is strictly prohibited. It shall not be a defence that caseworkers were mistaken about the age of the child. Exchange of money, employment, goods or services with anyone in return for favours is prohibited. Immediately report cases of sexual exploitation and abuse to the police, LWU and the village leader in a confidential manner.
- **Honesty and due diligence:** Caseworkers should, at all times, honestly perform their duties. They should always carry out their duties to the best of their ability and make every effort to fulfil their responsibilities.

vii. Roles and Responsibilities

Social service workforce

The social service workforce forms the critical human resource pillar of the child protection system. Referring to various professional and para-professional workers serving in the social system, the social service workforce shares a common goal to care for, support, promote and empower vulnerable people.³⁷ The social service workforce plays a crucial role in identifying risks, facilitating access to essential social services and supporting poverty reduction. In Lao PDR, multiple government organisations are mandated to provide child protection services. These include the Ministry of Labour and Social Welfare (MOLSW) for supporting vulnerable children, the Lao Women's Union (LWU) for handling cases of violence against women and girls, and the Lao People's Revolutionary Youth Union (LYU) for providing counselling services and receiving reports of cases of youth. Also, anyone who works with and for children and families, including NGO and CSO staff, can be considered as the human resources of the child protection system whose contributions can prevent, respond and identify child maltreatment risks.



CPN Structure

Chairperson of CPN
Deputy/ Village Chief
Vice Chairpersons of CPN
Village Labour and Social Welfare
Village Women Union
Members
Village Front for National Development
Village Security Unit
Director of School
Village Health Worker
Village Youth Union Secretary

At the village level, the Child Protection Network (CPN) plays a critical role as the eyes and ears of the community. Child Protection Networks (also known as Village Child Protection and Assistance Committees — VCPACs) came into being in Lao PDR in 2004 under the leadership of the Department of Social Welfare in the Ministry of Labour and Social Welfare (MOLSW) with support from UNICEF.

³⁷ Source: Guidelines to Strengthen the Social Service Workforce for Child Protection. UNICEF, New York, 2019

They were subsequently formally integrated into the child protection system under the “Ministerial Agreement on Establishment of Child Protection Networks” in 2017. According to MOLSW, as of 2021, CPNs have been established in 2,184 villages, approximately 26% of all villages in Lao PDR (#8,507). The establishment and strengthening of CPNs have been an important component in the national child protection strategy in Lao PDR. The Law on the Protection of the Rights and Interests of Children (2007) determines the roles of Child Protection and Assistance Committees (CPACs) to manage and monitor the situation of children through a network of committees at the village level. Article 36 of the Law identifies the Committees as having the following key duties:

- To receive reports on children who are at risk of harm and children in need of special protection.
- To assist children and families in need of special protection.
- To advise, monitor and regularly inspect the care and assistance given to children through various services and to collect statistics on children who are at risk of harm or in need of special protection.

Illustration of the CPN Roles and Responsibilities

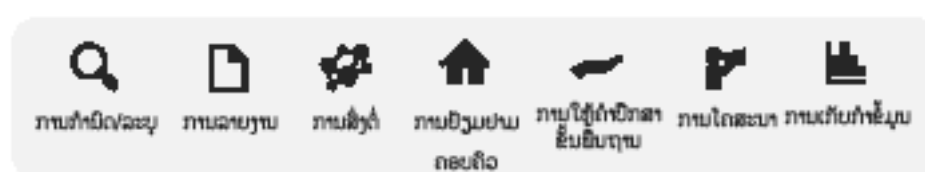


Table A: Child Protection Network (CPN) Members, Roles and Responsibilities ³⁸

CPN members	Roles and responsibilities
Village heads/ deputy village heads	• Act as leader of the CPN at the village level and ensure regular meetings, and maintain the minimum standards for CPN operations. • Oversee integration and implementation of child protection, rights and development issues in Village Development Committee meetings and relevant actions at the village level. • Ensure the well-being of children is monitored, and child protection work is integrated into the village’s annual work/development plan. • Call meetings as necessary to facilitate the work of the CPN or to address cases requiring CPN interventions. • Ensure cases beyond the remit of the CPN are referred to relevant authorities or service providers, including at the district level. • Mobilize resources for the implementation of child protection activities.

³⁸ Core Recommendations for Strengthening the Child Protection System in Lao PDR. Child Protection Assistance Committee, 2022



Ministry of Labour and Social Welfare (MOLSW) volunteer	• Act as a member of the village CPN committee and serve as the CPN Village Secretary and focal point or as a coordinator. • Collect data on children in the village and those needing special protection. • Monitor and follow up on child protection concerns with other CPN members. • Conduct family visits to help and support to deal with issues or report • Support MOLSW social workers on case follow-up/monitoring. • Support preparation of annual work plans and reports on CPN activities.
Village Mediation Unit (VMU) focal person	• Handle cases of children in conflict/contact with the law in alignment with relevant laws and regulations on child protection. • Monitor and record information on vulnerable children and children at risk. • Represent children or their caregivers in filing lawsuits. • Act as a focal point for coordination with related agencies at the village level to provide immediate assistance to children. • Raise child protection issues in their village during village cluster meetings. • Advocate and awareness raising about the child protection system within the village.
Lao Women's Union (LWU) focal person	• Undertake family visits to support them in accessing assistance to address child protection concerns. • Act as a referral point for children and receive reports on child protection concerns. • Identify and support families whose children are not attending school or are at risk of not receiving adequate care. • Support village-level mediation efforts to address child protection concerns. • Provide psychosocial and legal counselling services and accompany children and families through court process as necessary. • Conduct parenting education to equip parents with knowledge and skills for better care and protection of their children.



Lao Front for National Development (LFND) focal person	• Offer advice for establishing a village fund for child protection (both in terms of prevention and response). • Support advocacy efforts related to child protection, child rights and national laws and policies for child wellbeing. • Mediate between traditional beliefs and child protection standards as necessary. • Support the integration of child protection, child rights and well-being into village work and development plans. • Offer advice to children and families about how to prevent and respond to child protection issues, including how to seek assistance.
Lao Youth Union (LYU) focal person	• Organize developmental activities for youth and children, including sports, cultural activities, performing arts, discussion forums, etc. • Ensure that all activities take place in a way that promotes child rights, protects their well-being, promotes overall development and includes children with disabilities. • Encourage youth and children's participation and serve as volunteers in different village structures and activities. • Report potential problems to the LWU, VMU or village chief. • Refer children to different forms of support and services.
Village Security	• Support the timely investigation of all incidents or allegations of violence, abuse, neglect, exploitation, discrimination and vulnerability or crimes involving children. • Support awareness raising and understanding of laws and policies relating to children, including how issues will be addressed under the system. • Document and report child protection issues that arise in the community. • Act as a referral point and support for children and families to access services. • Monitor and support child safety and create an enabling environment in the village.

School Principal/ teachers	• Act as a member of the CPN committee and a link between the school and the village to ensure that the principles of child rights and responsibilities promoted in each setting complement and reinforce each other. • Ensure monitoring and implementation of child protection policies at school. • Monitor school attendance and coordinate with CPN volunteers to facilitate access to education for all children. • Seek assistance from other CPN members or service providers if suspected child protection cases arise. • Coordinate with the village CPN committee to assist children needing protection.
Village health volunteers	• Act as a member of the village CPN committee and coordinate health-related assistance to children needing protection and their families. • Promote and ensure every child under age five, especially newborns, are registered and have a birth certificate. • Coordinate with the village CPN committee and CPN volunteers to help children in need of special protection and their families.

Justice and law enforcement officials

The justice and law enforcement sector plays an essential role in ensuring that every child can access a justice system that is child-friendly, gender-sensitive and well-equipped to secure their rights. Children in contact with the law, including child offenders, child victims (predominantly female) and child witnesses, need special protection, care and focus. Collaboration between the justice and social welfare sectors is essential to ensure children's access to child-friendly justice services and a continuum of care. In Lao PDR, the Juvenile Justice Coordination Committee plays a vital role in coordinating and handling cases involving children. The members of the committee include the Ministry of Justice (MOJ), the Ministry of Public Security (MOPS), the People's Supreme Court (PSC), and the Office of the Supreme People's Prosecutor (OSPP).

The Village Mediation Unit (VMU) is established at the village level under the Ministry of Justice (MOJ). VMU is a local structure based on longstanding community mechanisms for dispute resolution promoted by the government in most villages in Lao PDR.³⁹ The VMU is responsible for handling civil and minor criminal disputes. Mediation is a voluntary process during which a neutral third person (a mediator) helps those involved in a civil or criminal dispute work towards an acceptable agreement for everyone involved. Mediation involving children is an alternative to

³⁹ Decision of the Minister of Justice No 304/ MOJ.



the formal justice system for child offenders. During mediation, the victim and the child offender meet in an informal setting to discuss the offence. It gives the victim an opportunity to describe how the offence affected him/her and allows the child offender to provide his/her side of the story. The mediator facilitates the discussion and development of a mutually acceptable outcome that addresses the harm caused.

Teachers and school staff

The Ministry of Education and Sports (MOES) staff, teachers and school administrators interact with children daily and often have first-hand knowledge of their circumstances, both at school and home, based on their attendance, performance in school and experiences they may share. The system should endeavour to improve the protection and well-being of children in schools. Teachers can also play a role in the early identification and reporting of children experiencing protection issues and either provide support themselves or refer children to those who can.

Health workers

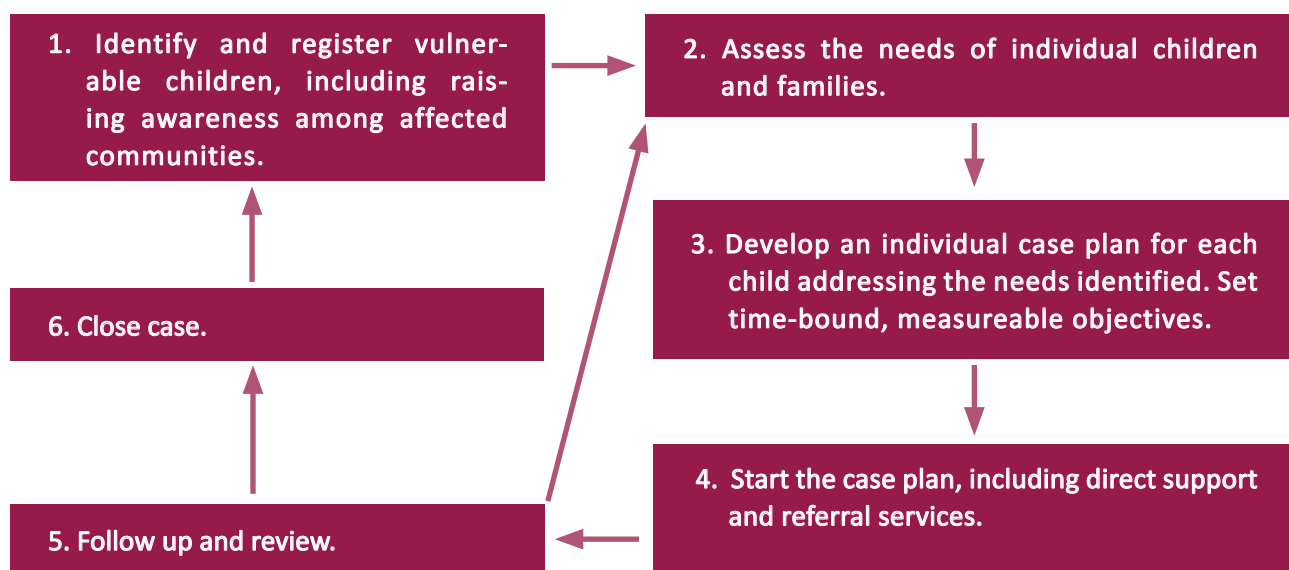
The Ministry of Health (MOH) staff, doctors, nurses and health centre staff are often the first officials who may encounter cases of abuse and violence against children, especially in severe cases when a child has been harmed physically or sexually. The health workers can play a role in the early identification and reporting of child abuse, provide immediate medical aid and refer children to those relevant service providers. They have a crucial responsibility of immediately reporting cases of physical and sexual violence. They can also play an essential role in providing mental health and psychosocial support (MHPSS) to distressed children and families.

Chapter II

Case Management Process

This chapter explains the process of case management and explores key actions, steps and considerations:

Chart A. Case Management Process



i. Identification and Registration

Case management begins when there is reporting about child protection concerns. Child Protection Network (CPN) members have the primary responsibility of receiving any suspected case of violence, abuse, neglect, exploitation and other child maltreatment in villages. Cases can also be reported to the provincial/district Labour and Social Welfare and the provincial/district Lao Women's Union concerning women and girls. Local police and security officials also need to be alerted.

Cases can come to the attention of these authorities in many ways:

- A child may tell the authorities.
- Someone else from the family or a neighbour may tell authorities what a child has told them or that they strongly believe a child has been abused.
- A child may show signs of physical injury for which there appears to be no satisfactory explanation.
- A child's behaviour may indicate to you that it is likely that he or she is being abused.
- The authorities' awareness of or suspicion about an abuse due to something disclosed by a third party who suspects or has been told of the abuse.

- Something in the behaviour of the child or in the way a parent(s) or caregiver interacts with a child alerts the authority.

Confidentiality is key: Confidentiality is difficult, especially when abuse is disclosed. A person making a report may prefer not to disclose the name or identity of the perpetrator. The report receiver should always be aware of how to deal with all allegations or suspicions of abuse in line with the best interest of the child.

When a case is identified, the CPN should determine the level of risks and proceed with the case assessment by following the below steps:

- Step 1: Fill in the Child Registration Form (pX). This collects basic information about the child's name, age, sex, ability, and parental situation. Document the contents of the report and the concerns/risks identified.
- Step 2: Based on the concern/risks identified, check whether other sectors need to be involved in initial case handling/response, such as Health, Police and Education.
- Step 3: Report to the Social Welfare Officer at the provincial/district Labour and Social Welfare for official case registration.

Each case should be registered with a unique case reference number for easy monitoring. The case reference number can have the following information: Location, name of the office, year, and number. For example:

- ATP-POLSW-2022-01: Attapeu Province, Provincial Office of Labour and Social Welfare, the year 2022 and case number 1.

Table B: Codes for Locations

Code	Full name	Code	Full name
ATP	Attapeu	ODX	Oudomxay
BK	Bokeo	PSL	Phongsaly
BKX	Bolikhamxay	SRV	Saravan
CPS	Champasak	VTE	Vientiane Province
SVK	Savannakhet	VTE Cap	Vientiane Capital
HPN	Houaphanh	XBY	Xayaboury
KHM	Khammouane	XK	Xekong
LNT	Luang Namtha	XKH	Xienkhouang
LPB	Luang Prabang	XSB	Xaysomboun

Table C: Examples of Codes for Organizations

Code	Full name	Code	Full name
MOLSW	Ministry of Labour and Social Welfare	PLWU	Provincial Lao Women's Union
POLSW	Provincial Office of Labour and Social Welfare	DLWU	District Lao Women's Union
DOLSW	District Office of Labour and Social Welfare	MOPS	Ministry of Public Security
CPN	Child Protection Network	MOJ	Ministry of Justice
LWU	Lao Women's Union	LYU	Lao Youth Union

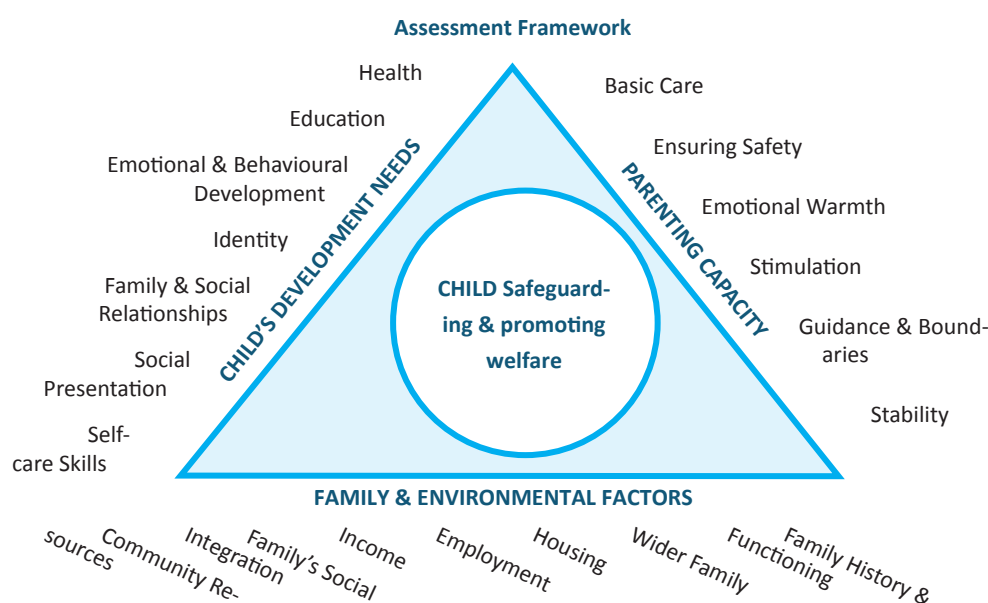
ii. Assessment

The case assessment should occur within 24 - 48 hours of case reporting and registration. It aims to assess the protection risks that a child is in, such as the degree of threats, harms and risks, types of cases and family/living situation. Home visits and in-person interviews with the child, family members, neighbours, or other relevant stakeholders should be made to probe the problem. Please see Tips for Home Visits (Page 63) for more details..

- **Step 1:** Conduct a home visit (if needed, make the visit together with other members of the CPN or go together with the District Social Welfare Officer) and fill in the Child Assessment Form (Page 37) and Family Assessment Form (Page 41)

When assessing the child and family, the following areas need to be considered:

Diagram: Assessment Framework ⁴⁰



⁴⁰ Framework for the Assessment of Children in Need and their Families, Department of Health on behalf of the Controller of Her Majesty's Stationery Office, UK.

When a child is in danger and needs immediate protection and care, the following steps should be followed:

- **Step 1:** Contact police and security focal persons and remove the child from danger.
- **Step 2:** Place the child in a safe place, such as LWU protection shelters, licensed foster care families, CSO shelters, registered alternative care homes, etc.
- **Step 3:** Provide counselling support to the child and direct the child to appropriate social welfare services.
- **Step 4:** Ensure the child's psychosocial well-being and follow up on referrals and service delivery.

Table D: Classification of Cases by Level of Urgency for Children²

Type of risk/harm	Level 1 Significantly harmed. Urgent response and frequent follow-up are required within 24 hours, and weekly follow-up is needed.	Level 2 Harmed. Response and follow-up are required within 3 days, and weekly follow-up is needed.	Level 3 At risk of harm. Monitoring is required within 7 days, and monthly follow-up is needed.	Level 4 No present risk. No further action is required, and the case can be closed.
Physical abuse	• Serious injury (bone break, burn, scars, severe bruise, etc.) • Threats to disability • Case of infants and toddler • Child is self-harming • Attempt to suicide	• Excessive corporal punishment • Visible signs of abuse (bruises, scars, etc.) and threats to injury	• Threats to injury • Use of corporal punishment	• No violence • Perpetrators are no longer with the child
Sexual abuse and child marriage	• Any sexual contact between a child and an adult • Child is pregnant • Child is married	• A child has been sexually abused in the past but did not receive support	• A child is promised to be married	• There are no sexual harm factors present

² Adopted from the Case management Standard Operating Procedures for Children in Need of Care and Protection, Ministry of Gender, Children and Social Protection of the Republic of Ghana (June 2018). (<https://www.google.co.kr/url?sa=t&rct=j&q=&esrc=s&source=web&cd=&ved=2ahUKEwjVrd6apsf3AhUEpLYBHQjZC80QFnoECAoQAQ&url=https%3A%2F%2Fwww.mogcsp.gov.gh%2F%3Fmdocs-file%3D2753&usg=AOvVaw2ge0mjLJevBX-FOq0S8FA4>)

Psychological abuse	• A child is being persistently isolated, belittled, or humiliated by a caregiver	• Occasional belittling, isolation or humiliation	• A child is treated differently than other siblings, and parents are hostile toward the child	• Parents and children received the support, and factors causing the emotional harm are addressed.
Neglect	• Serious injury or illness, including malnutrition • Child-headed household	• Lack of parental supervision • Inadequate basic care • Failure to protect • The child is often left alone • The child has to undertake tasks beyond his/her developmental capacity	• A child is taken care of by the elderly	• The child's basic needs are met • The child is adequately taken care of by the parents • A child is living in a safe environment
Highly vulnerable children	• Abandoned children under five • Unaccompanied children under 5 • Separated children under 5 with unknown family	• Unaccompanied children under 12 • Separated children under 12 with unknown family • Children with disabilities • Children living on the street • Children with HIV/AIDS	• Children living in IDP shelters • Children in COVID-19 quarantine camps	• A child is in good care, and the situation has been monitored for several weeks with no issues arising
Child Victims of Trafficking	• A child is trafficked • A child is forced to marry • A child works in hazardous conditions (mining sector, dangerous factories, bars, etc.)	• Underaged children are forced to work • A child is working long hours and has dropped out of school	• Parents want to send the child to work, not to school	• A child is not working • A child is reintegrated into school

iii. Case Planning

Within a week of the case assessment, case planning should take place. Case planning includes specific interventions to address the issues identified from the case assessment. The case plan should be tailored to the individual child, keeping in mind his/her situation, needs and wishes in line with the 'best interest of the child' principle. The interventions should be clear and practical based on the local context.

- **Step 1:** The District Social Welfare Officer, with the help of the CPN, should set up 'case conference(s)' with the child, family and other relevant stakeholders.
- **Step 2:** Mutually agree on the goal. (Why) (i.e., to reintegrate a child back to school, to find the family of a child living in an orphanage and return him/her to home, to rehabilitate a child who experienced physical violence at home, etc.)
- **Step 3:** Using the Case Planning Form (pX), the District Social Welfare Officer should develop specific, measurable, achievable interventions with clear responsibilities and timeframe for monitoring. (How, Who, What, When, and Where).
- **Step 4:** Using the Service Mapping Tool (pX), map available resources, services and contacts in the community that can contribute to implementing the case plan.
- **Step 5:** Agree on the implementation modality and monitoring plan with the family and share it with the family if applicable.
- **Step 6:** Clearly explain to the family and the child about the implementation and monitoring plan in a child/disability-friendly and age-specific language to manage expectations.

An important consideration is **child participation**. Children are aware of their experiences; therefore, their views, opinions and wishes should be fully respected when developing a case plan. When communicating with the child, the Social Welfare Officer and the CPN should consider the child's age and level of development. Also, a child's wishes should not harm or keep the child in a harmful situation. The principle of the 'Best Interest of the Child' should be taken into account.

Case Implementation

Based on the case plan, the District Social Welfare Officer and the CPN can start implementing specific actions to support the child in need of help.

- **Step 1:** Establish a clear point of contact. The child should know where to call and reach out to when in need of help.
- **Step 2:** Based on the Referral Request Form (pX) and the Referral Pathway (pX), engage sectors (Justice, Health, Education, Social Protection, Agriculture, etc.) for service referrals and delivery.
- **Step 3:** Ensure continuous support to the child and families. The frequency of support and follow-ups should be decided case by case.
- **Step 4:** The District Social Welfare Officer can seek funding from the Provincial Labour and Social Welfare Office to facilitate case management.
- **Step 5:** Monitor, follow up and reporting.

The boxes below present some good examples of referral services in Lao PDR

Peaun Mit



CHILDREN ARE PLAYING AT THE PEUAN MIT'S TEMPORARY SHELTER © UNICEF LAOS.

Peuan Mit (“Good Friends” in Lao language) operates a holistic programme for children, youth and caregivers living on the streets and in marginalised communities in Vientiane Capital and Luang Prabang Province. For young children, Peuan Mit pursues reintegration into education and into caring family environments. Older children and youth separated from family are provided short-term shelter at the Transitional Home, while Peuan Mit works in the background to reunite them with their families. Marginalised youth are linked with vocational training and stable employment. The program also works with caregivers to build their income-generation capacity.

- www.friends-international.org/in-laos
- www.unicef.org/laos/stories/care-and-protection-violence-time-covid-19
- www.unicef.org/laos/stories/story-souk

Box A. Example of Services for Referral: Emergency Shelter for Vulnerable Children

Lao Women's Union Counselling and Protection Center for Women and Children



LWU HELPLINE POSTER © LWU AND UNICEF LAOS/2021

The Lao Women's Union (LWU) is responsible for assisting victims of violence and trafficking through its reporting and counselling helplines (Call number: 1362) and Counselling and Protection Center for Women and Children, located in Vientiane Capital and Luang Namtha Province. LWU staff provides legal aid free of charge for victims of violence and rehabilitates survivors to reintegrate into society. LWU counsellors and staff work closely with all sectors for timely and quality referrals. LWU volunteers are members of the Village Mediation Union, Village Development Committees and the Child Protection Network in villages.

- www.unicef.org/laos/stories/LWU-Helpline

Box B. Example of Services for Referral: Mental Health Support for the Youth

Lao Youth Union Helpline (1554)

The Lao People's Revolutionary Youth Union (LYU) is responsible for empowering young people as agents of change to provide assistance in protection issues such as child marriage and adolescent pregnancy, and runs toll-free 24/7 mental health and psychosocial counselling helpline (call number 1554) for adolescents and young people. Through the helpline, LYU counsellors can identify cases and make appropriate referrals.

- LYU Facebook: www.facebook.com/LaoYouthUnion
- www.unicef.org/laos/reports/manual-youth-and-adolescents-counselling

Box C. Example of Services for Referral: Social Protection

The Mother and Early Childhood Grant (MECG) by MOLSW

The Mother and Early Childhood Grant (MECG), implemented by the Ministry of Labour and Social Welfare (MOLSW) is part of a joint initiative between UNICEF, ILO, and UNCDF aimed at improving access to health and child protection services in Lao PDR for pregnant women and children. There are a total of 2,596 beneficiaries in 46 villages of Sanamxay district, 20 villages of Phouvong, Attapeu province, and 70 villages in Nong district, Savannakhet province enrolled under the MECG, which is a cash transfer pilot for pregnant women and young children aimed at improving the uptake of vaccinations and access to antenatal and postnatal as well as child protection services, including pregnant women and children under six months of age.

- www.unicef.org/laos/press-releases/mother-and-early-childhood-grant-first-cash-transfer-beneficiaries-under-initiative

Box D. Example of Services for Referral: Parenting Education and Economic Empowerment

Saving for Change by the Lao Women's Union



LWU PARENTING EDUCATION SESSION IN ATTAPEU PROVINCE © UNICEF LAOS/2020

Supported by KOICA and UNICEF, the Lao Women's Union has been implementing the parenting education with the Saving for Change (SfC) model in Attapeu province since 2020 to empower the displaced families in Sanamxay shelters to prevent domestic violence and other harmful practices against women and children. This is done through parenting education on positive discipline, gender equality, care and protection of children with disabilities, and supporting financial savings and training to build economic resilience at the family level. In 2021, 190 parents and caregivers (164 female and 26 male) actively participated in the SfC activity.

- Lao PDR - Providing parenting support for young children through the 'Love and Care for Every Child' initiative (p27): www.unicef.org/eap/reports/ending-violence-against-children-east-asia-and-pacific



Box E. Example of Services for Referral: Inclusive Education

Association for Autism, Lao PDR

Association for Autism is a registered non-profit association funded by parents of children with autism in Lao PDR. It is a self-help group founded in 2005. The Association for Autism (AfA) represents the interests of persons with autism spectrum disorder (ASD) in Lao PDR by promoting their rights, improving their access to appropriate services, and providing them and their families with education, therapy, and training to equip them better to live full and productive lives.

- <http://www.laoautism.org/index.html>

Box F. Example of Services for Referral: Alternative Care

SOS Children's Village

Children who have lost parental care can find a home in one of the 14 SOS families in Lao PDR. The children grow up with their sisters and brothers in an environment of care, respect, and security.

As the children grow into young adults, they can move into SOS youth programmes. With the support of qualified professionals, they are guided through this new stage of their lives, as they start vocational training courses, attend higher education and start looking for work. The youths are encouraged to develop perspectives for their future, learn to shoulder responsibility and increasingly make their own decisions.

- <https://www.sos-childrensvillages.org/where-we-help/asia/laos/vientiane>

Box G. Example of Services for Referral: Police

Police Helpline: 1300

1300 is hotline number operated by the Ministry of Public Security. Victims of trafficking and people who witness or suspect trafficking cases can call the helpline. The helpline staff will provide response services by reporting the case immediately to the relevant authorities domestically and internationally to rescue and provide support to the victims and take legal proceeding against the perpetrators.

Box H: Example of Services for Referral: Legal Aid

Legal Aid

Under the Decree on Legal Aid (no 77/Govt, dated 28 Feb 2018), legal aid is a free legal service for the poor, disadvantaged, children in need of special protection and perpetrators who require legal assistance as required by the law. People with disabilities, women and child victims of violence and victims of trafficking can also seek legal aid. The Ministry of Justice's digital legal aid platform allows people to access free legal aid services wherever they are: <https://legal-aid.moj.gov.la>. At the village level, the Village Mediation Unit (VMU) provides free or low-cost mediation services.

iv. Case Review

Throughout the case implementation, the District Social Welfare Officer and the CPN should regularly follow up, review and monitor the implementation progress.

- **Step 1:** Initial home visits can be made every month to every quarter to monitor the family situation and the well-being of the children. The review meetings should take place together with the child, families and concerned stakeholders. Please see **Tips for Home Visits (Page 63)** for more information.
- **Step 2:** Each home visit and review session should be well documented to track the progress using the **Case Review Form (Page 57)**.
- **Step 3:** Agree on the progress made and exchange experiences about what went well and what could be improved. When needed, the case plan can be adjusted. In this case, the adjustment of activities should be clearly documented in the original case plan.
- **Step 4:** Case review meetings can occur every six months Depending on the progress.
- **Step 5:** When multiple sectors are involved, the district/provincial social welfare officer can organise a **case or coordination meeting** to share the latest situation of the case and coordinate the case plan implementation. Please see the Case Conference Form **(Page 55)**.



v. Case Closure

Based on the recommendations of the CPN, officials from multiple sectors involved, and in agreement with the child and family, the case can be closed when the district/provincial social welfare officer is confident that the goal of the case plan is achieved, namely when the living environment of the child is safe, and the child is in good care. When it is agreed to close the case, the District Social Welfare Officer should fill in the **Case Closure Form** (Page 60) and submit it to the Provincial Social Welfare Officer

- **Step 1:** When ready, discuss with the CPN, concerned officials and child/families about the case closure.
- **Step 2:** Determine whether the goal of the case plan is achieved.
- **Step 3:** If yes, the District Social Welfare Officer should fill in the **Case Closure Form** (Page 60) and submit it to the Provincial Social Welfare Officer. The case files should be safely stored in the safe box/cabinet of the district/provincial social welfare officer.
- **Step 4:** Support the child and families to be prepared for case closure.





Chapter III

Case Management Tools, Resources and Forms

In this chapter, you can find standard tools, resources and forms for case management:

- i. Case Registration Form
- ii. Child Assessment Form
- iii. Family Assessment Form
- iv. Case Planning Form
- v. Referral Request Form
- vi. Referral Pathway
- vii. Service Mapping Tool
- viii. Case Conference Form
- ix. Case Review Form
- x. Case Closure Form
- xi. Tips for Home Visits
- xii. Do's and Don'ts
- xiii. CPN Roles and Responsibilities
- xiv. Code of Conduct
- xv. CPN Data Collection and Summary Record-Keeping Form
- xvi. District and Provincial Data Collection and Reporting Form

i. Case Registration Form

Form 1

CASE REGISTRATION FORM				
		Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.		
To be filled by the caseworker		Case ref. #		
Date and time		Signature		
Name				
Office address				
Phone number				
Reporting received from				
Method of reporting	Telephone ☐	In-person ☐	Helpline ☐	Other ☐ :
Child and family details				
Child's first name		Child's family name		
Date of birth and age:	Sex: Male ☐ Female ☐			
Child's address:	Contact info:			
Name of the mother:	Mother's address:			
Occupation of the mother:	Contact info:			
Name of the father:	Father's address:			
Occupation of the father:	Contact info:			
If the child is living with other family members and/or cared for by another caregiver other than biological parents, please write the information:				
Protection risks: Has the child faced any protection concerns in the past, or is there any concern that the child might face in the near future?				



Physical violence ☐	Neglect ☐	Abandonment ☐	Trafficking ☐
Double-orphanhood ☐	Single-orphanhood ☐	Sexual abuse ☐	Child labour ☐
Street children ☐	Serious health issue ☐	Disability ☐	HIV/AIDS ☐
Out of school ☐	Child marriage ☐	Early pregnancy ☐	Child offender ☐
Please describe detailed protection risks and child vulnerabilities: 			
Proposed actions: 1. Does the case need further investigation? Yes ☐ No ☐ 2. Does the case need to be referred to other services? If yes, to: _____ 3. Please describe actions to be taken: 			

ii. Child assessment form

Form 2

CASE ASSESSMENT FORM			
			
Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.			
To be filled by the caseworker		Case ref. #	
Start date of the assessment		End date of the assessment	
Name			
Office address			
Phone number		Signature	
Assessment of the child's development and family situation			
1. Health: This section can be completed by discussing with the parents and village health workers based on available documentation and observations.			
Child's immunization	Yes ☐	No ☐	Description:
Health insurance	Yes ☐	No ☐	Description:
Disability status	Yes ☐	No ☐	Description:
How many meals does the child have? Does the child feel the amount of food is enough?	Number ()	Description:	
Does the child have any diseases?	Yes ☐	No ☐	Description:
If yes, does the child receive regular treatment?	Yes ☐	No ☐	Description:
Describe other relevant information.			

2. Education: This section can be completed by discussing with the parents, teachers or village education focal persons based on available documentation and observations.					
Level of education completed.					
Does the child regularly go to school?		ປົກກະຕິ ☐	ບໍ່ປົກກະຕິ ☐	Name of the school:	
School address				Contact info:	
If the child does not regularly go to school, why?					
3. Household living conditions: This section can be completed by discussing with the parents, caregivers, family members and other relevant stakeholders.					
Describe the general condition of the home.					
Describe the financial situation of the household.					
Does the house have adequate access to clean water facilities?		Yes ☐	No ☐	Description:	
Does the house have adequate access to sanitation facilities?		Yes ☐	No ☐	Description:	
Family members living together.		Mother ☐	Father ☐	Grand-mother ☐	Grandfather ☐
		Siblings ☐ (how many:)			
		Other caregivers:	i.e. uncle, auntie		Child-headed households ☐
		Description			



Is the child expected to do household chores or engage in income-generating activities? Are these expectations reasonable for the child's age/abilities?	Yes ☐	No ☐	Description:
4. Behaviour assessment: This section can be completed by discussing with the child, parents and caregivers.			
What is the child's general emotional status?	i.e., happy, unhappy, angry, fearful, etc.		
Does the child have any behavioural issues?	i.e., disobedience, wetting bed, stealing, lying, self-harming, drug abuse, etc.		
Does the child have social skills and gets along well with other children?			
5. Protection assessment: This section can be completed by discussing with the parents, child, family members and other relevant stakeholders (i.e. neighbours and village leaders) based on the documentation available and observations.			
Protection risks: Has the child faced any protection concerns in the past, or is there any concern that the child might face in the near future?			
Physical violence ☐	Neglect ☐	Abandonment ☐	Trafficking ☐
Double-orphanhood ☐	Single-orphanhood ☐	Sexual abuse ☐	Child labour ☐
Street children ☐	Serious health issue ☐	Disability ☐	HIV/AIDS ☐
Out of school ☐	Child marriage ☐	Early pregnancy ☐	Child offender ☐
Please describe detailed protection risks and child's vulnerabilities:			



6. Overall recommendations			
Does the case need immediate action? Please explain:			
Does the case need mid- and long-term actions? Please explain:			
Referrals to other services to be made	Police ☐	Education ☐	Health ☐
	Provincial/district LSW ☐	LWU ☐	NGO/CSOs ☐
What are the wishes of the child?			
Recommended placement	Return to mother ☐	Return to father ☐	Return to both parents ☐
	Placement in extended families ☐	Residential care home ☐ (Name	Other ☐ :

iii. Family Assessment Form

Form 3

 FAMILY ASSESSMENT FORM 			
Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.			
To be filled by the caseworker		Case ref. #	
Name		Date	
Title		Office	
Office address			
Contact info		Signature	
Child(ren), parents, carers and significant others			
Name of the child			
Date of birth		Sex	Male ☐ Female ☐
Home address			
Contact info			
Name of the mother			
Home address			
Contact info			
Name of the father			
Home address			
Contact info			



Details of the other children in the household	Name	Date of birth	Relationship to the child
Other caregivers	Name	Relationship to the child	Address
Other significant people NOT living in the house	Name	Relationship to the child	Address
Family assessment (Please copy the family book or write down the family mapping of the case)			
1. Basic situation assessment			
Who has parental responsibility?	Who are the primary carers? Are there any Court Orders on who the child or young person has contact with or lives with?		
Communication needs of the family	What is the child/parents' first language? Is there a need for an interpreter? Does the child use other forms of communication, such as Sign Language?		



Additional information regarding ethnicity or cultural heritage:	Consider ethnic and cultural issues which need to be understood. Do the child and family understand their culture and heritage well? Has the child or family suffered any discrimination?		
Legal status & immigration status of the family:	Does the family have legal citizenship?		
Other considerations	i.e., Birth registration		
2. Professional relationship: Details of professionals involved in the child and family's lives (school teacher, village health volunteer, etc.)			
Name	Role	Address	Contact info
3. Family situation			
Family history and functioning	i.e. Illness, bereavement, violence, parental substance misuse, criminality, anti-social behaviour, culture, size and composition of the household and daily absent parents or significant others.		
Wider family	What are the formal and informal support networks from extended family members and others? (i.e., broader caring responsibilities)		



Housing	Are there any issues with current family home, i.e., water, heating, sanitation facilities, sleeping arrangements? Are there reasons for homelessness?
Employment and income	Income/benefits, work and shifts, employment, understanding why unable to work, debts, effects of hardship. Does the family receive the government financial support?
Family and social integration	Write any issues with family relationships, support or relationships from the wider community and neighbours.
Community resources	Are there daycare centres, schools, transport, leisure and entertainment places, protection shelters, peer groups (i.e., parenting or youth clubs), etc.?
4. Parenting capacity	
Basic care	Write information about how the child is cared for, such as providing food, drink, warmth, shelter, appropriate clothing, and personal and dental hygiene.
Ensuring safety	Write information about safety and health environment at home and provision of adequate supervision.



Emotional warmth	Do the parents provide the child with a stable, affectionate, stimulating family environment, praise, encouragement, and secure attachments?
Stimulation	Do the parents know and use positive activities, encourage learning, and undertake learning activities together?
Guidance and Boundaries	Do the parents use positive behaviour patterns, effective and appropriate discipline techniques, and avoid over-protection to guide the child/children?
Stability	Is there physical and emotional stability within the household environment? i.e., frequency of house or school moves; frequency in changes to significant relationships.
5. Any other important considerations	
Is there any other information that parents, carers, children, young people, or the practitioner need to include?	
Consider what support, if any, has been provided before. Did the family find it helpful? If not, why?	



What are the views of the family? When reintegrating children from an institution to the home, are the parents willing to accept the child?

6. Analysis

What are the needs and risks for the child?

What are the family's strengths and protective factors?

In your view, can/does the family provide a suitable living situation for the child?

iv. Case Planning Form

Form 4

CASE PLANNING FORM			
			
Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.			
Child's detail		Case ref. #	
Child's first name		Child's family name	
Date of birth and age		Sex	Male ☐ Female ☐
Child's address			
Contact info			
Background information			
Date of developing the case plan			
Persons involved in the case plan development (when appropriate, ensure to involve the child)	Name	Relationship to the child	
Case planning			
1. Actions required to improve the well-being of the child			
Education	Does the child have any specific educational needs? Does the child require specialised education needs or care in the case of child/children with disabilities?		



Health	Does the child need any specific medication or access to health services? Does the child have a disability? Does the child require any assistive devices or specialised support in their physical development?
Household living conditions	Does the child require basics needs in current living conditions? Does the child require to be placed in a home with specific amenities? Would the child//adolescent/family need any support in finding employment or require economic support?
Behaviour characteristics	In case the child shows signs of challenging behaviour, is any specific physiological or parenting support required? Does the child need counselling?
Protection	What intervention is required if the child is suffering from violence, abuse, exploitation or neglect?
Other considerations	i.e., Birth registration
2. Views of the concerned people	
What are the wishes of the child?	
What are the wishes of the family?	



3. Action plan			
No.	Action	Responsible	Timeframe

Report prepared by			
Name		Date	
Position		Address	
Contact info.		Signature	

To be filled by the supervisor			
Name		Date	
Position		Address	
Contact info.		Signature	



v. Referral Request Form

Form 5



Lao People's Democratic Republic
Peace, Independence, Democracy, Unity, Prosperity

District:.....

Village:.....

No /

Village....., date.....

Referral Request Form

To: Name of the organization

Subject:

- Reference is made to the case reference number.

(Name of village or organisation that will refer the case)_____. We are referring (name of client) _____ to you to receive relevant services. The reason for the referral and the services required is as follows: _____

Thank you for your help with this referral. We look forward to your prompt feedback to assist with the management of this case.

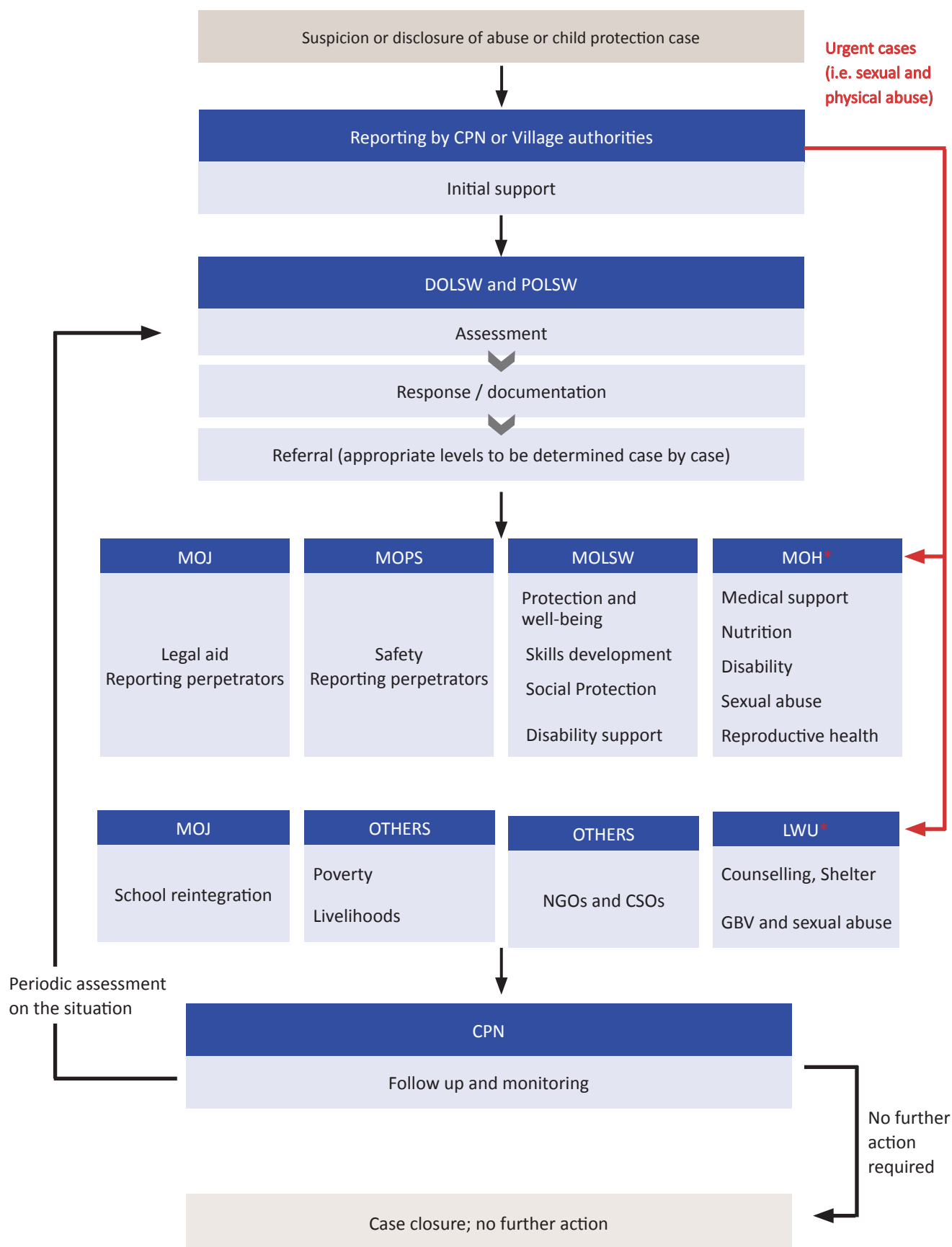
Sincerely,

From

(signature and name)

Contact number: _____name

vi. Referral Pathway





vii. Service Mapping Tool

Other information						
Registration status						
Disability yes /no. Mild, moderate, severe						
Services provided						



Types (Government, CSOs, etc.)	Emergency, short, medium, or long-term placements	Number of children served	Age range & gender of children served



Contact info						
Manager						
Location						
Name						

viii. Case Conference Form

Form 6

CASE CONFERENCE FORM			
			
Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.			
Child's detail		Case ref. #	
Child's first name		Child's family name	
Date of birth and age		Sex	Male ☐ Female ☐
Child's address			
Contact info			
Case conference summary			
A. Summary			
Date			
Participants	Name	Organisation	
B. Agenda			



C. Discussion			
A. Next step			
No.	Action	Responsible	By when
Report prepared by			
Name:		Organization:	
Contact info:		Signature:	

ix. Case Review Form

Form 7

CASE REVIEW FORM			
 Note: keep written reports and other information confidential. Only share information with persons directly involved in the case. 			
Basic information		Case ref. #	
Child's first name		Child's family name	
Date of follow-up/review			
Purpose			
Location of the follow-up	Child's home ☐	Office ☐	By phone ☐ Other:
Participants (ensure to involve the child when appropriate)	Name		Relationship to the child
Child's case plan review			
A. Child's overall well-being			
Education			
Health			
Household living condition			
Behaviour characteristics			
Protection			



Child's current care arrangement	i.e., adoption, foster care, institution, kinship care, reintegration to biological parents, etc.		
B. Overall analysis			
Since the assessment, are there any significant positive/negative changes in the child and family situation?			
Are there any action points from the case plan that have not been implemented yet? Why?			
C. View of the concerned people			
What is the view of the child about the case plan implementation? Is the child happy and satisfied? Are there any areas that can be supported further?			
What is the view of the family about the case plan implementation? Is the family happy and satisfied? Are there any areas that can be supported further?			
D. Write any actions that need to be adjusted or introduced to the case plan.			
No.	Action	Responsible	By when



E. Next review			
Next target review date			
Report prepared by			
Name:		Organization:	
Contact info:		Signature:	

x. Case Closure Form

Form 8

CASE CLOSURE FORM			
			
Note: keep written reports and other information confidential. Only share information with persons directly involved in the case.			
Basic information		Case ref. #	
Child's first name		Child's family name	
Date of case closure			
Participants (ensure to involve the child)	Name	Relationship to the child	
Case closure checklist			
1	The child/adolescent is in a safe, stable environment, free of violence, harm or abuse.		☐
2	Completed all the case management forms and went through all the steps of the case management process		☐
3	Ensured that all objectives under the care plan have been completed or are under the responsibility of a competent individual		☐
4	Conducted at least three additional follow-ups to ensure that the child/adolescent is happy within the home and is accessing appropriate community services		☐
5	Ensured that the child has access to appropriate health services (if applicable)		☐



6	Ensured that child is enrolled in pre-primary, primary or secondary education (if applicable)	☐
7	Provided the family with counselling and parenting support to address any behavioural characteristics	☐
8	Engaged with local authorities to ensure that the child has a support network	☐
9	For children/adolescents with a disability: • Linked to all available resources within the community • (Foster) parents have the necessary skills to provide support to and interact with the child/adolescent	☐
If all boxes are not checked, but the case is still closed, explain why:		
Reasons for case closure		
Case goal achieved		☐
The child/family moved to another district/province/country, and the case is transferred.		☐
Child deceased		☐
The child turned 18 and does not need/want child protection services.		☐
Others:		☐
Final comments		



Report prepared by	
Name:	Organisation:
Contact info:	Signature:
To be filled by the supervisor	
Name	Date
Position	Address
Contact info.	Signature

xi. Tips for Home Visits

Tips for Home Visits

GENERAL ETTIQUETTE

- Bring something with you to engage the children whenever possible, such as games or books.
- Introduce yourself.
- Let the family know who you are and your role as CPN.
- Wait to be invited into the home.
- Recognize the rules in the home (i.e. take off shoes at the door).
- Reassure the family that you are there to be supportive and to help them to be successful as a family unit.
- Consider safety as CPN. For example, is there anything in the home that makes you feel unsafe?

DURING THE VISIT

- Relate to the family using cultural practices, cultural celebrations, community/village values, family and cultural expectations.
- Use your knowledge of the community and community members to help the child and family address needs and problems.
- Trust your instincts, if something doesn't feel quite right.
 - It's okay to ask questions as long as you are not judging.
 - It's okay to leave the home if you feel unsafe.

BE OBSERVANT

- Notice how the child and family are getting along with each
- Notice any obvious bruising or discomfort of the child
- Notice changes in behavior such as withdrawal or isolating
- Notice how the parent disciplines/punishes
- Does there seem to be family cohesion? Is the family united and working together to help each?
- What are the medical needs, food/nutrition needs, changes in income, safety problems in the home?
- Be prepared to listen. Don't do all of the talking



xii. Do's and Don'ts

AFTER THE VISIT



- Be consistent: visit regularly whenever possible and respect the family boundaries, i.e. no visit during dinner time or when family has guests.
- Use good ethical practice: i.e. don't share information with your family, friends or others; don't accept gifts; don't give money.
- Follow up: If you provide information on resources or supports, check-in with the family to see if they accessed the resources. Were the resources helpful? Does an alternative resource or social support need to be identified?

DOs

- Stay calm.
- Listen and hear what the person is saying.
- Give time and space to the person so they can say what they need to say.
- Reassure the child (or other individual) that s/he has done the right thing to report this situation.
- Record the reported abuse in writing.
- Refer the case to the appropriate service provider.

DON'Ts

- Do not panic.
- Do not judge or talk down to the child or parents.
- Do not make a child repeat the story unnecessarily.
- Do not try and remember everything to write it down later. Write it down as you are being told of the situation.
- Do not try to deal with a situation of abuse alone.
- Do not bring the child into your own house. Avoid meeting children in other inappropriate places (hotel, bar, etc.). In case a child needs to be removed immediately from his/her home, CPN needs to contact local authorities, LWU and MOLSW at the district and provincial levels who can recommend placing the child in an emergency safe home.

xiii. CPN Roles and Responsibilities

Suggested roles and responsibilities in data collection and case management among the Child Protection Network (CPN):

Responsible Person	Task
Any CPN member	Conduct a home visit.
Any CPN member	Fill in individual case forms.
Any CPN member	Coordinate and manage individual cases, including referrals. Inform village authorities of cases.
Head of CPN	Fill in the summary form for record-keeping.
Head of CPN	Fill in the summary form for submission.
Village chief (in case the Head of CPN is the village chief, this step can be skipped)	Review individual case forms.
Village chief (in case the Head of CPN is the village chief, this step can be skipped)	Review the summary form for submission.
Village chief Backup: Head of CPN	Submitting individual case forms and quarterly/bi-annual/annual forms to district Labour and Social Welfare

xiv. Code of Conduct

The Social Service Workforce, including the village Child Protection Network (CPN), in their interactions with children and families and how they carry out their responsibilities, their conduct must always be of the highest standard, and their behaviour should set a positive example for other community members.

The following standards outline how this commitment should be carried out. They cover both their relationship with individual children and families and how they carry out their broader responsibilities as members of CPN. No list can be exhaustive, but the principles underlying the Code of Conduct should be applied to any new challenges that arise.

Best Interests of the Child – The social service workforce, including the CPN member, should always work in the best interests of the children who have suffered or are at risk of abuse.

Personal Behaviour – The personal behaviour of the CPN member should be consistent with the principles and ethos of the child protection service, especially in terms of her/his relationships with and treatment of children. These standards apply to all their dealings with children, including those within their own family, and not just those related to their work with the CPN.

Confidentiality – The CPN member must treat any personal information that s/he has access to, whether about a child, individuals who report their concerns, alleged perpetrators of violations or one of their colleagues, as confidential and not share it with third parties outside of the CPN group who do not have a legitimate reason for having access to it.

Respect and Collaboration – The CPN member should treat other members or individuals who bring their concerns to the CPN respectfully and work with them in a spirit of cooperation.

Equality – The CPN member must treat everyone involved in the team equally, irrespective of religion, gender, disability, ethnicity, socioeconomic background, etc.

Honesty – The CPN member will, at all times, honestly perform their duties.

Due Diligence – The CPN member will always carry out their duties to the best of their ability and will make every effort to fulfil their responsibilities.

Conflict of Interest – The CPN member must always try to avoid conflicts of interest. Should any possibility of a conflict of interest arise, they must immediately report and discuss this with the village chief, DOLSW or POLSW or with a professional social worker. In this context, it would not usually be appropriate for a CPN member to be involved in responding to an incident or violation where the victim or the alleged perpetrator is a member of their family or a close friend.

Prevention of Sexual Exploitation and Abuse – Any attempted and/or actual act or form of sexual harassment is strictly prohibited that could result in physical, sexual or psychological harm or suffering to individuals, especially women and children. Engaging in sexual exploitation or abuse of beneficiaries under any circumstances is strictly prohibited. It shall not be a defence that CPN was mistaken regarding the child's age. Exchange of money, employment, goods or services with any-



one in favour of returns in connection with the case is strictly prohibited. Immediately report cases of sexual exploitation and abuse to the police, LWU and the village leader in a confidential manner.

Reporting Concerns – If a member has concerns about the behaviour of another member of the CPN group and/or believes that they are in breach of this Code of Conduct, they should raise their concerns confidentially with the village chief, DOLSW or POLSW. If this is not possible, CPN should raise their concerns with the MOLSW at the national level.

Date:

Name:

Title:

Location:

Signature:

xv. CPN Data Collection and Reporting Form

Instructions

- The Head of CPN should prepare and submit one reporting form per village on a monthly, quarterly, and annual basis to the District of Labour and Social Welfare.
- The village chief should review the form before submission. If the Head of CPN is the same person as the Village Chief, please sign on both “Head of CPN” and the “Village Chief”.
- Please record the name and basic information of vulnerable children and ✓ the vulnerability that applies.
- A child may have more than one vulnerability, if so please tick all the boxes that apply.
- If the vulnerability of the child is not listed, please ✓ the “others” column and explain in the “Remarks” column.

Definitions

Double-orphaned child means a child whose parents are both deceased.

Half-orphaned child means a child whose father or mother is deceased.

Neglect means the parents or guardians persistently fail to meet a child’s basic physical or psychological needs.

Abandonment means the parents’ or guardians’ decision to have no role in the child’s life permanently.

Physically abused children are children who have been beaten or tortured by their parents, guardians or other persons.

Sexually abused children mean children who have been raped or are victims of sexual relationships or obscenity.

Children with disabilities means children who have long-term physical, mental, intellectual or sensory impairments which may hinder their full and effective participation in society on an equal basis with others.

Child victim of trafficking means any child who has directly suffered in his or her physical or mental health, dignity, freedoms or his/her property resulting from all forms of trafficking in persons.

Children affected by HIV/AIDS are children infected with HIV/AIDS, children who have been half-orphaned or orphaned due to HIV/AIDS, or children living with an HIV/AIDS-positive family member.

Child labour means children under twelve who work in non-hazardous or hazardous places for more than the hours stipulated under the labour laws and other relevant regulations.

Child marriage is defined as a marriage of a girl or boy before the age 18.

Early pregnancy means a girl before the age of 18 becomes pregnant.

Child offenders means children who are subject to criminal proceedings: as suspects, defendants or convicted persons.

Out-of-school children means children who are at school age but are not in school.

Village..... District..... Province

of all children in the village (Girls/Boys)
(Example) 130 girls/ 100 boys

of vulnerable children (Girls/Boys)
(example) 1 girl/1 boy

Reporting Frequency: (V) Quarterly () Annual Reporting Period:.... Quarter 1, 2021 (Jan-March)

TYPES OF VULNERABILITIES		Remarks		She was referred to a district health and LWU					
		Others (please specify)				V			
		Out of School Children		V					
		Child offenders							
		Early pregnancy							
		Child marriage							
		Child labour							
		Children affected by HIV/AIDS							
		Child victim of trafficking							
		Children with disability							
		Sexually abused children		V					
		Physically abused children							
		Abandonment							
		Neglect				V			
		Half-orphaned				V			
		Double-orphaned							
Case No.	Child's Full Name	Age	Sex	Ethnic					
No. 1/2 021	(Example) Noy Sengxay	13	F	Lao-Tai					
No. 2/2021	(Example) Sak Thonddy	6	M	Lao-Tai					

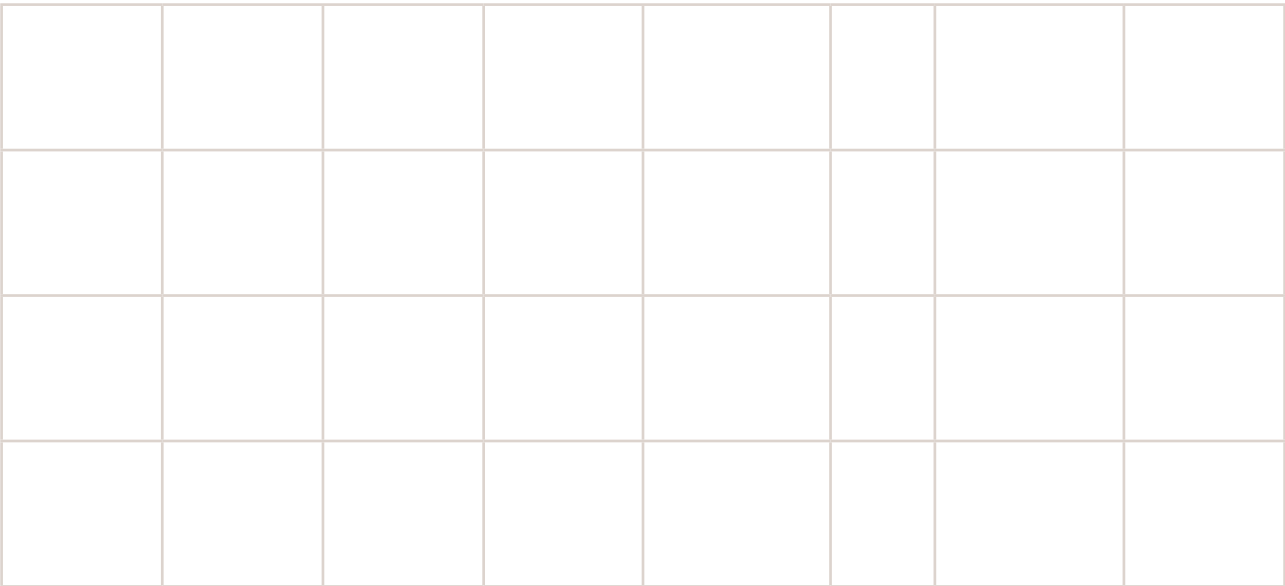
Prepared by (name) / Head of CPN Village Chief (name)..... Signature:.....

xvi. Village Summary Record Keeping Form

Instructions

-  This form is for **record-keeping at the village level**.
-  The Head of the CPN in the village should summarise the total number per quarter or year based on the “CPN Data Collection Form on Vulnerable Children”. Please ensure the data consistency on the “# of all children in the village” and the “# of vulnerable children in the village” between this form and the CPN Data Collection Form on Vulnerable Children.
-  For “Reporting Frequency”, put “Monthly/Quarterly/ Annual”.

Reporting Period	Reporting Frequency	# of All Children in the Village		# of Vulnerable Children in the Village			Prepared by
		Total	Girl/Boy	Case no.	Total	Girl Boys	
[example] Jan—Mar 2021	Quarterly	230	130/100	No.1/2021 - No.2/2021	2	1 / 1	Name/Title





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United For Every Child: 50th Anniversary of UNICEF in Lao PDR
ສາມາດນຳພາລາວໄປສູ່ໂລກທີ່ດີກວ່າ: 50 ປີ ຮ່ວມ ປັບປຸງ ປະຊາກອນ ລາວ